

From ghost patients to a fake vendor network in a national health insurance scheme

Risk-based review of claims and procurement controls for a national public health insurance authority.

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From ghost patients to a fake vendor network in a national health insurance scheme

A national public health insurance authority needed an independent view of whether its claims and procurement controls were protecting public funds. Following payments beyond the initial findings exposed a vendor network that was later shut down.

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CLIENT TYPE	ROLE	APPROACH	SCOPE
National public health insurance authority	Senior Analyst, EY	Risk-based review	Claims & procurement

EXECUTIVE SUMMARY

SITUATION A national public health insurance authority wanted an independent view of its claims and procurement controls.	COMPLICATION Fraud was spread across enrolment, referrals, billing and provider procurement, each hiding the others.
APPROACH Risk ranking, analytics on the full claims population, and fieldwork tracing payments through intercompany deposits.	IMPACT Hundreds of suspect claims flagged. A fake vendor network shut down. Four control failures prioritised for remediation.

4 fraud schemes found, from enrolment through to procurement	6 stages of the money trail tested, from enrolment to vendor payment	Hundreds of suspect claims and ghost-patient registrations flagged	Shut down the fake vendor network found in the review
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01 · CONTEXT

Every stage of the claims cycle exposed public funds to leakage

National health insurance schemes pay empanelled hospitals for treating registered beneficiaries. Each step, from enrolment to admission, pre-authorisation, claim, payout and the hospital's own purchasing, is a point where money can be taken. The authority wanted an independent view of where its controls were failing and who was benefiting.

02 · RISK-BASED APPROACH

A risk-based approach concentrated fieldwork on the highest-risk providers and processes



1 Planning

Mapped the claims cycle and the providers' procurement cycle. Ranked the risks by likelihood and impact, and put fieldwork time into the highest-risk providers and processes rather than spreading it evenly.

2 Data analytics

Tested the full claims population for red flags: identity mismatches, impossible treatment patterns, clusters of claims from one referral source, and directors who also appeared on vendor records.

3 Fieldwork

Checked patient records against registrations, followed payments through intercompany deposits and counterparty records, and verified whether vendors really existed and operated.

4 Root cause

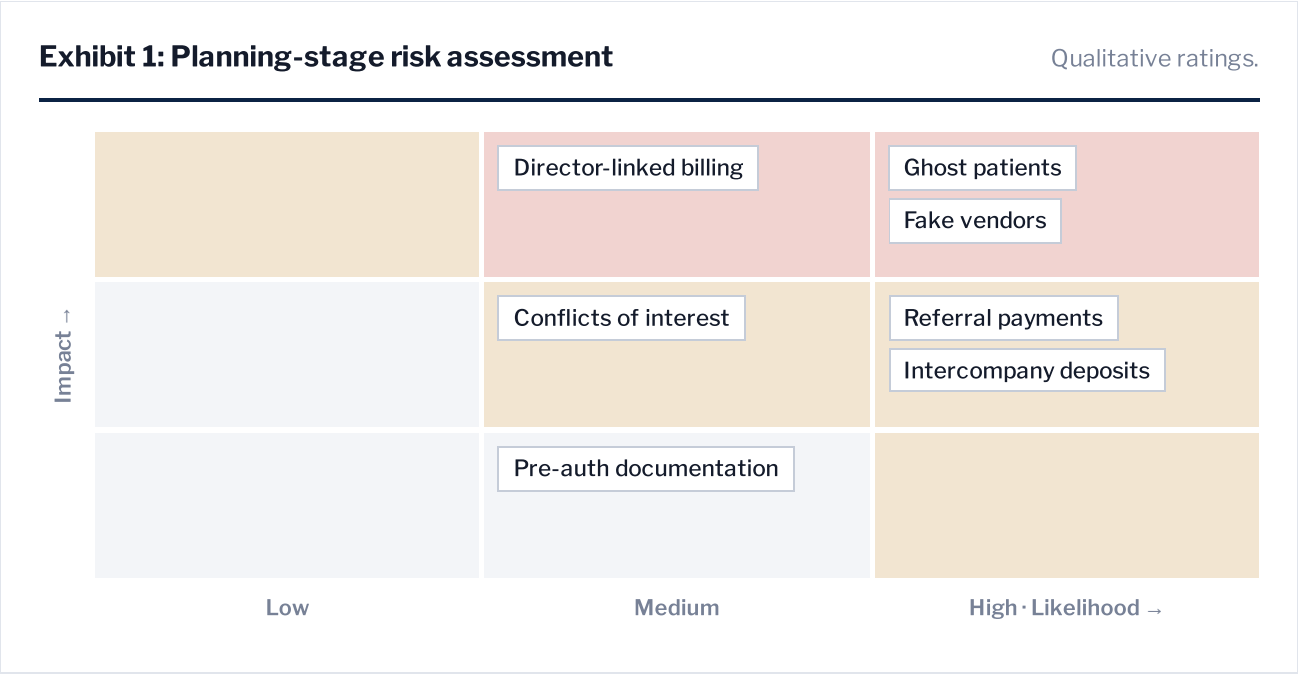
For each finding, identified the control that failed (verification, segregation, disclosure or vendor due diligence) and explained how it was bypassed.

5 Reporting

Rated the findings, supported each with evidence, and gave practical recommendations. Worked with the authority on what to do first.

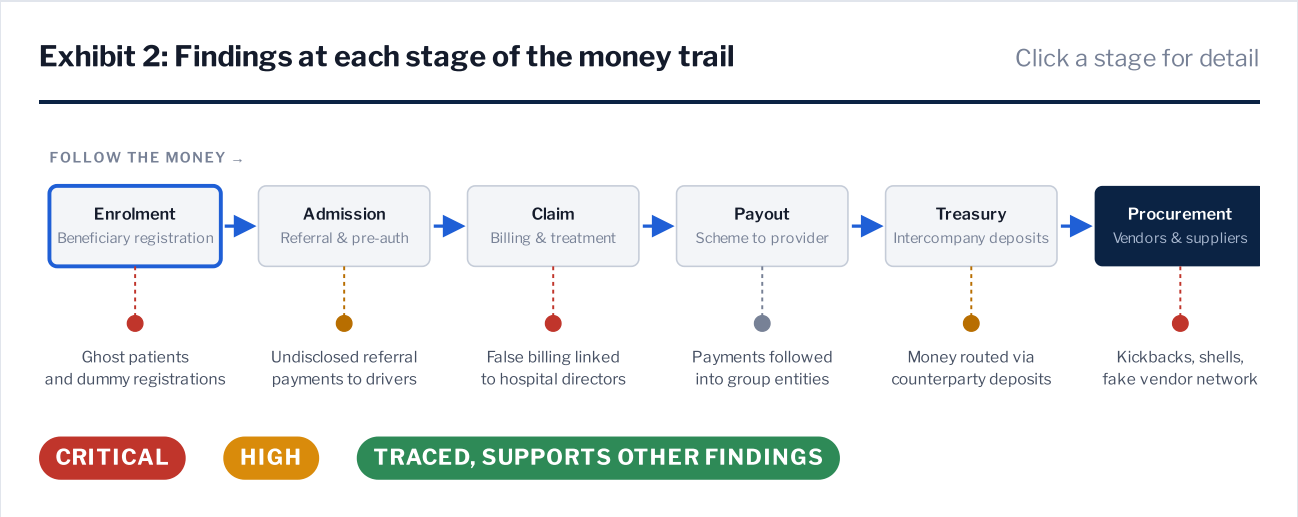
03 · RISK ASSESSMENT

Claims fraud and procurement fraud were rated the highest risks at planning



04 · MONEY TRAIL

Following payments past the first findings exposed the procurement network



05 · FRAUD SCHEMES AND COMPLIANCE ISSUES

Four fraud schemes were each traced to a specific control failure

SCHEME	HOW IT WORKED	CONTROL THAT FAILED	RAG RATING
Ghost patients & dummy registrations	Beneficiaries who didn't exist, or weren't treated, were registered and claimed for.	Identity verification at enrolment and admission	CRITICAL
Director-linked false billing	False claims traced to entities connected to the hospital's directors.	Conflict-of-interest declarations; claim audit	CRITICAL
Undisclosed referral payments	Payments to ambulance drivers to steer patients to particular hospitals, never disclosed.	Referral monitoring; disclosure requirements	HIGH
Procurement kickbacks & fake vendors	Money moved through intercompany deposits to shell companies and a fake vendor network.	Vendor due diligence; segregation of duties in procure-to-pay	CRITICAL

CRITICAL

Red

HIGH

Amber

MODERATE

Green

06 · IMPACT

The vendor network was shut down and the authority received a prioritised control plan

The fake vendor network found in the review was later shut down. The findings gave the authority evidence to act against the providers involved, and a list of control changes to stop the same schemes happening again across its network.

Recommendations

RECOMMENDATION	ADDRESSES	PRIORITY
Verify beneficiary identity at enrolment and again at admission	Ghost patients and dummy registrations	HIGH
Require disclosure of referral arrangements and director conflicts of interest	Referral payments; director-linked billing	HIGH

RECOMMENDATION	ADDRESSES	PRIORITY
Introduce vendor due diligence and procure-to-pay segregation of duties for empanelled providers	Kickbacks and fake vendors	HIGH
Run analytics-based red flags for ongoing claims monitoring	All schemes	MEDIUM

KEY TAKEAWAYS

- 1 Fraud at one stage often funds or hides fraud at another. Follow payments across the whole cycle, not one process.
- 2 Referral and procurement arrangements are claim-fraud enablers and belong in scope.
- 3 Full-population analytics points fieldwork to where the risk actually is.

RELEVANCE TO UAE ROLES

Relevance to the UAE market

Mandatory health insurance in Dubai and Abu Dhabi has the same exposure points: provider fraud, referral incentives and upcoding. DHA and DoH Abu Dhabi expect insurers, TPAs and providers to run fraud controls. This review is directly relevant to payer fraud units, insurer internal audit and healthcare compliance roles.